

Standard Operating Procedure

Proactive Patient Safety Risk Analysis

«Hospital Name» — Supports PSQ.1.e

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By Quality Coordinator Name: _____ Signature: _____			
Approved By Medical Superintendent Name: _____ Signature: _____			

1. Purpose

This SOP describes how «Hospital Name» carries out proactive analysis of patient safety risks — before an incident happens — and turns the findings into real improvements.

2. Scope

Applies to at least one patient safety risk analysed proactively every year, covering both clinical and non-clinical processes and areas.

3. Responsibility

Safety Committee: selects the risk/process to analyse, runs the analysis, and tracks resulting improvements.

Patient Safety Officer: coordinates the analysis and reports outcomes to the Safety Committee.

4. Procedure

1. Select a process or risk for proactive analysis — prioritise a high-risk, high-volume, or error-prone process (e.g. IV pump programming, medication look-alike risk, handoff between shifts).

2. Choose an analysis tool suited to the question:

- Failure Mode and Effects Analysis (FMEA) examines a process step by step for every way it could fail, then prioritises the failure modes with the greatest potential impact. Use it when you want to redesign a specific process before it fails.
 - Hazard Identification and Risk Analysis (HIRA) identifies hazards and rates each by likelihood and severity, so the highest-priority risks get attention first. Use it for a broader area (e.g. an entire ward) rather than one process.
 - Hazard Vulnerability Analysis (HVA) identifies the emergencies the hospital could face and their direct and indirect effect on operations. Use it for disaster/business-continuity style risks, not routine clinical processes.
3. Worked example (FMEA on IV pump programming): list every step of programming an infusion pump; for each step, list ways it could fail (e.g. wrong rate entered, wrong drug library selected); score each failure mode for severity, likelihood of occurrence, and likelihood of detection; multiply the three scores to get a Risk Priority Number; redesign the steps with the highest numbers first (e.g. add a mandatory second-person check for high-risk drug libraries).
 4. Document the tool used, the findings, and the risk ranking in the Proactive Risk Analysis Record (Record 7).
 5. Implement improvements based on the findings — do not simply document the risk and stop there.
 6. Confirm at least one such analysis has been completed in the current year before the annual programme review.

5. Records

Supported by the Proactive Risk Analysis Record (Record 7) of the PSQ.1 Required Records pack.

6. Related Documents

Policy on Patient Safety Programme (HCO/PSQ/POL/01) — PSQ.1 policy, Section 3.5 (proactive risk analysis)

7. Review

Reviewed annually, or immediately if a proactive analysis reveals a gap in this procedure.

8. References

Institute for Healthcare Improvement. (n.d.). Failure Modes and Effects Analysis (FMEA) Tool. <https://www.ihi.org/library/tools/failure-modes-and-effects-analysis-fmea-tool>

California Hospital Association. (n.d.). Hazard Vulnerability Analysis. <https://calhospitalprepare.org/hazard-vulnerability-analysis>